Dose-dense AC → weekly paclitaxel

National Cancer Control Programme (NCCP) 00485 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Your treatment and access plan

Four AC treatments (doxorubicin 60 mg/m² and cyclophosphamide 600 mg/m²) 14 days apart, followed 14 days after the last AC by 12 weekly paclitaxel 80 mg/m² doses, subject to recovery. The planned sequence spans about 20 weeks.

STEP 1

Before treatment

full blood count (FBC), kidney and liver tests at baseline and before each cycle, with the unit confirming weekly paclitaxel test requirements. Review current illness and past dose delays separately.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. A port is not automatically required; insertion and removal are separate procedures with individual instructions.

STEP 3

On the treatment day

Confirm the medicines due that day, any premedicines and the reviewed blood results. Tell staff immediately about pain, burning, rash or breathlessness.

STEP 4

After the visit

No routine take-home infusion pump is required for this regimen. Keep line-care, urgent-contact and next-review instructions.

Visit length

AC and weekly paclitaxel have different visit and supportive-medicine plans. The unit confirms the complete appointment duration.

Line care

A PICC or port has its own care plan. Report fever, redness, swelling, pain, leakage or line damage; do not remove or adjust it yourself.

Treatment changes

A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

Support for this visit

granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) is required for every dose-dense AC cycle. Prescribe product, dose and timing separately under the approved local plan. First paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment (NCCP suggested table).

Sources: NCCP 00485 v6 · official regimen · eviQ · implanted port patient information · eviQ · PICC patient information. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.